



Armada Kalamunda Group

Admission of Mental Health Consumers Referred to Inpatient Services Guideline

1. Purpose

Ensure timely admission of mental health consumers to the most suitable available mental health inpatient bed. Ensure an effective handover between referring clinicians and ward clinicians receiving the consumer.

The Inpatient Bed flow coordinator is the individual responsible for the coordination of inpatient beds, excluding those in CRHTT, at a given time. This role is contactable on the AH MH CNS phone and is routinely covered by the inpatient ANUM group and the Afterhours Mental Health CNS.

2. Guidelines

2.1 Step 1: Process for Admission

- The referring mental health clinician contacts the relevant Bed flow coordinator; Inpatient bed flow coordinator or EMHS CRHTT CNS as appropriate and provides a clinical handover in line with [WA Health Clinical Handover Mandatory Policy MP0095](#).
- Written documentation, is to be sent via internal email: ahs.mhinpatientref@health.wa.gov.au or ArmadaleCRHTTReferrals@health.wa.gov.au as required and as per [AKG Admission and Assessment Documentation Requirements](#).
 - For referrals originating within AKG this requirement can be met by sending an email to ahs.mhinpatientref@health.wa.gov.au or ArmadaleCRHTTReferrals@health.wa.gov.au as required identifying the consumer's name, UMRN, type of bed required and confirmation that documentation as per [AKG Admission and Assessment Documentation Requirements](#) is complete and available in electronic format (i.e. WebPSOLIS or DMR).
- The referring mental health clinician enters a bed request on Enterprise Bed Management (EBM). If they are unable to access EBM then the relevant Bed flow coordinator can be requested to do this.
- The relevant Bed flow coordinator will discuss the referral with the allocated treating team/DMO and enter bed request on EBM if this has not occurred. The Inpatient bed flow coordinator will liaise with the referring clinician to organise a timely admission if accepted or advise if declined with rationale.
 - If the referring clinician wishes to dispute a referral that has been declined then the dispute resolution process outlined in the [East Metropolitan Health Service \(EMHS\) Mental Health Patient Flow and Bed Capacity Framework](#) is to be followed.

Before referencing this guideline document please ensure that you have the latest version from the [Policy Library](#) information hub page.

Consideration will be given regarding suitability of referrals to AKG in the context of existing consumer/staff cohort where concerns regarding therapeutic benefit, confidentiality, any relevant legal action, or other contraindications are present.

Decisions regarding specific ward allocation of accepted patients and their suitability i.e., for patients under the Mental Health Act (2014) to be admitted and managed on an open ward is the responsibility of the accepting treating Consultant in discussion with the ANUM responsible for that area or if afterhours the On-Call Consultant in discussion with the Afterhours Mental Health CNS.

Where there are indications of intoxication or physical health issues, the referring clinician will provide a medical clearance or if the consumer is being referred from AKG Community Mental health team member then they are to be referred to the Armadale Health Service Emergency Department (AHS-ED) to obtain medical clearance.

The Bed flow coordinator, in consultation with the Consultant Psychiatrist/On Call Consultant, will arrange the prioritisation of beds.

When a bed is not available, the referring mental health clinician is to liaise with the EMHS Patient Flow Coordinator Mental Health to locate a bed at an alternative health service, as per: [EMHS Mental Health Patient Flow and Bed Capacity Framework \(Summary Sheet\)](#)

2.2 Admissions Accompanied by Police

Damage to a consumer's property and personal effects may occur during the transfer process from the community environment to the Health Service when a consumer is transferred under the Mental Health Act.

When AHS clinicians request Police assistance and the police act under direct instruction from mental health staff, AHS will be responsible for any damage sustained to the consumer's property and personal effects during this process.

All admissions escorted by Police to high dependency unit (HDU) Yorgum ward and Banksia Ward, will enter via the Leschen Unit ambulance emergency entrance.

Whenever possible, prior to arrival, the referring mental health clinician is to call the relevant Bed flow coordinator to advise of their pending arrival. Transport services and Police are directed by inpatient staff to the airlock for gun storage if they wish to utilise it.

Inpatient staff will be available to receive the consumer upon arrival, however, should delays occur, where possible, the consumer is to remain in the vehicle.

3. Legislative context

The following documents are required to give effect to this policy:

- [Western Australia Mental Health Act 2014](#)
- [HDWA: Mental Health Bed Access, Capacity and Escalation Statewide Policy](#)

4. Supporting information

The following documents support the implementation of this policy:

- [WA Health State-wide Standardised Clinical Documentation for \(SSCD\) for Mental Health Services OD 0526/14](#)
- [Mental Health Framework for Statewide Standardised Clinical Documentation: 'Triage to Discharge'](#)
- [Office of Western Australia Chief Psychiatrist: Clinicians' Practice Guide to the Mental Health Act 2014](#)
- [Armadale Mental Health Service National Outcome and Casemix Collection \(NOCC\) Procedure \(MEN-PRO-0276-18\)](#)
- [East Metropolitan Health Service \(EMHS\) Mental Health Patient Flow and Bed Capacity Framework](#)

5. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

Standard 5: Action 5.4 The health service organisation has systems for comprehensive care that:

- Supports clinicians to develop, document and communicate comprehensive plans for patients care and treatment
- Provide care to patients in the setting that best meets their clinical needs
- Ensure timely referral of patients with specialist healthcare needs to relevant services
- Identify, at all times, the clinician with overall accountability for a patient's care

Standard 5: Action 5.7 The health service organisation has processes relevant to the patients using the service and the services provided:

- For integrated and timely screening and assessment
- That identify the risks of harm in the 'Minimising patient harm' criterion

Standard 5: Action 5.10 Clinicians use relevant screening processes:

- On presentation, during clinical examination and history taking, and when required during care
- To identify cognitive, behavioural, mental, and physical conditions, issues, and risks of harm
- To identify social and other circumstances that may compound these risks

Standard 5: Action 5.12 Clinicians document the findings of the screening and clinical assessment processes, including relevant alerts, in the healthcare record

National Standards for Mental Health Services (2010):

Standard 10: Criterion 10.3 The entry process to the MHS meets the needs of its community and facilitates timeliness of entry and ongoing assessment

6. Document owner

Service Director Mental Health

Enquiries relating to this guideline may be directed to:

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7. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V6	31/10/2023	31/10/2026	Minor amendment

8. Authorisation

This guideline has been authorised and issued by the Service Director Mental Health

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